



RESEARCH ARTICLE

Adult Attention-Deficit/Hyperactivity Disorder and the Ethics of Medicalization: Between Recognition and Diagnostic Expansion

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ABSTRACT

Adult Attention-Deficit/Hyperactivity Disorder has become one of the most controversial developments in contemporary psychiatry. Once primarily understood as a childhood behavioral condition, Attention-Deficit/Hyperactivity Disorder is now increasingly conceptualized as a lifespan neurodevelopmental disorder whose symptoms may persist into adulthood. This transformation has contributed to the expansion of adult diagnoses and stimulant prescriptions, while simultaneously raising ethical and conceptual concerns regarding diagnostic expansion, medicalization, and pharmaceuticalization. The growing recognition of adult Attention-Deficit/Hyperactivity Disorder reflects not only changing clinical frameworks, but also broader transformations in how contemporary societies understand attention, productivity, emotional regulation, and self-management. This paper develops a historical and conceptual analysis of the evolution of Attention-Deficit/Hyperactivity Disorder and its relationship to the medicalization of attentional and behavioral differences across the lifespan. Using conceptual analysis as its primary methodological approach, the article examines major shifts in the understanding of Attention-Deficit/Hyperactivity Disorder—from early moral and organic models to contemporary neurodevelopmental frameworks and neurodiversity perspectives. The paper argues that these transformations progressively expanded the interpretive boundaries of psychiatric diagnosis and reinforced pharmaceutical approaches to attention and self-regulation. In this sense, adult Attention-Deficit/Hyperactivity Disorder is best understood as a hybrid phenomenon: simultaneously involving legitimate clinical recognition of persistent impairments and broader sociocultural processes of medicalization and diagnostic expansion. Finally, the article develops a critical ethical interpretation of pharmaceuticalization and contemporary psychiatric classification, emphasizing the conceptual tensions surrounding the limits between pathology and ordinary variations in human behavior.

Keywords: Adult Attention-Deficit/Hyperactivity Disorder, Medicalization, Diagnostic expansion, Neurodiversity, ADHD

1. Introduction

Adult Attention-Deficit/Hyperactivity Disorder (hereafter: “ADHD”) has become one of the most significant and controversial developments in contemporary psychiatry. Once understood primarily as a childhood behavioral condition associated with hyperactivity and school disruption, ADHD is now increasingly conceptualized as a lifespan neurodevelopmental disorder whose symptoms may persist into adulthood. This transformation has contributed to a substantial increase in adult diagnoses and stimulant prescriptions, while simultaneously generating ongoing debates regarding overdiagnosis, diagnostic expansion, and the medicalization of everyday difficulties related to attention, self-regulation, and productivity.

The growing recognition of adult ADHD raises important ethical and conceptual questions. On the one hand, many individuals experience persistent impairments that were historically neglected or misunderstood, and clinical recognition may provide meaningful therapeutic benefits. On the other hand, the expansion of diagnostic categories into broader areas of adult life risks blurring the distinction between pathology and normal variation, particularly within highly performance-oriented social environments. In this context, the rise of adult ADHD cannot be understood exclusively as a scientific discovery or diagnostic refinement but must also be examined in relation to broader processes of medicalization and pharmaceuticalization.

This paper argues that the historical evolution of ADHD contributed to the progressive medicalization of attentional and behavioral differences across the lifespan. By examining major conceptual shifts in the history of ADHD—from early moral and organic models to contemporary neurodevelopmental frameworks—this article explores how changing diagnostic paradigms expanded the interpretive boundaries of pathology and reinforced pharmaceutical approaches to attention and self-regulation. The paper further argues that adult ADHD should be understood as a hybrid phenomenon: simultaneously involving genuine clinical recognition and broader sociocultural processes of diagnostic expansion.

Methodologically, this paper employs conceptual analysis to examine the historical evolution and normative implications of ADHD classification. Rather than evaluating the empirical validity of ADHD as a clinical condition, the article analyzes how changing conceptual frameworks progressively redefined the boundaries between pathology and normal variation. In this sense, the paper focuses on the ethical and conceptual assumptions underlying psychiatric classification, particularly regarding medicalization, pharmaceuticalization, and the expansion of diagnostic categories into adult life.

The article proceeds in three stages. First, it examines key conceptual transformations in the historical development of ADHD. Second, it analyzes the relationship between these transformations and processes of medicalization and pharmaceuticalization. Finally, it develops a critical ethical interpretation of adult ADHD as both a clinically meaningful and socially constructed diagnostic category.

2. The conceptual evolution of the Attention-Deficit/Hyperactivity Disorder

a. EARLY CONCEPTUALIZATIONS

The earliest descriptions of, or related to, what is now recognized as ADHD were centered on childhood populations.

It was George Still's 1902 series of lectures that most closely resembled the modern concept of ADHD. He conceptualized the disorder primarily as a deficit of “moral control” independent of intellectual impairment, linking attentional difficulties and impulsive behavior to failures of volitional self-regulation rather than to cognitive incapacity. His model combined developmental, behavioral, and moral dimensions, anticipating later concerns regarding self-regulation and attention.

During the early twentieth century, the emergence of the Brain Damage model shifted attention toward organic explanations of hyperactive behavior, particularly following observations of children affected by encephalitis epidemics. Hyperactivity and attentional problems were increasingly interpreted as consequences of neurological dysfunction, reinforcing a biological understanding of the disorder and indirectly opening the possibility of symptom persistence across the lifespan. Although the model did not explicitly address the persistence of symptoms into adulthood, such a course could be inferred from its emphasis on structural or organic etiologies. The underlying assumption was that biological damage would remain stable over time, potentially sustaining symptomatology across the life span.

When compared with Still's earlier model, both frameworks sought to identify the etiology for the observed symptoms. However, a key difference lies in scope: the Brain Damage model focused narrowly on organic explanations, while Still's formulation integrated volitional and developmental components. Additionally, the Brain Damage model emphasized an absence of clear diagnostic criteria and introduced considerable heterogeneity due to its broad and at times vague categorization.

b. THE HYPERKINETIC SYNDROME AND THE DSM-II

During the 1950s, the concept of the Hyperkinetic Syndrome emerged through the work of Laufer and colleagues^{1,2}. During the 1950s, the Hyperkinetic Syndrome model emphasized hyperactivity as the central feature of the disorder, while also associating it with impulsivity, attentional difficulties, low frustration tolerance, and behavioral maladjustment. Unlike earlier organic models, this framework focused more on observable behavioral symptoms and developmental immaturity than on identifiable brain lesions.^{1,2}

As this model gained traction, the concept of Minimal Brain Damage began to decline, giving way to more homogeneous diagnostic distinctions based on measurable clinical deficits. Within this shift, hyperactivity was prioritized as the most visible behavioral symptom of the disorder^{1,2}. Regarding prognosis, they hypothesized

that symptoms of the Hyperkinetic Syndrome might diminish over time. The etiological premise was that hyperactivity resulted from a “maturational lag” of the central nervous system, with improvements emerging alongside cortical maturation and increased motor control ¹⁻³.

Based on these developments, the “Hyperkinetic Reaction of Childhood” was incorporated into the DSM-II listed under “Disorders Usually First Evident in Infancy and Childhood” ⁴. The DSM-II included a brief descriptive paragraph: “a disorder characterized by excessive activity, restlessness, distractibility, and short attention span, which tends to diminish in adolescence”.

Laufer’s model, reflected in DSM-II, replaced earlier organic explanations with a more descriptive behavioral framework. Nevertheless, its lack of operational criteria and its exclusive focus on childhood would influence later conceptual debates on ADHD.

c. WENDER’S MINIMAL BRAIN DAMAGE MODEL AND DOUGLAS’S ATTENTION DEFICIT MODEL

During the 1970s, two parallel conceptual models of ADHD emerged: Paul H. Wender’s theory of Minimal Brain Damage (MBD), and Virginia Douglas’s Attention Deficit model. Both were particularly relevant for their focus on the long-term prognosis of children with hyperactivity, explicitly recognizing the persistence of symptoms into adulthood. Each model is examined below.

Wender reformulated earlier brain dysfunction models by emphasizing the persistence of attentional and impulsive symptoms into adulthood and proposing neurochemical rather than purely structural explanations. His work represented an important transition toward later neurobiological models of ADHD and contributed to the growing recognition that the disorder could persist beyond childhood.⁵ Wender argued that MBD should not be limited to child psychiatry, as its cardinal symptoms persisted into adulthood. Moreover, he hypothesized that these symptoms could evolve or transform into other psychiatric conditions over time.

Later, Wood et al. (1976), from Wender’s team, extended this model to adults, seeking to characterize persistent MBD symptoms and assess whether adults exhibited similar responses to stimulant and antidepressant treatment. Selection criteria included the presence of symptoms such as impulsivity, irritability, inattention, psychomotor restlessness, and emotional lability, in the absence of schizophrenia, affective disorders, brain syndromes, or intellectual disability ⁶.

Wender’s model represents a conceptual “bridge” between earlier structural models and future neurobiological approaches. It marked a transition from anatomical hypotheses to neurochemical ones and foreshadowed future recognition of ADHD’s heritability and persistence into adulthood ⁷. This model was also among the first to explicitly propose that stimulant treatment might benefit adults as well as children. And, despite its innovation, the model’s breadth of symptoms

overlapped with multiple modern diagnostic categories (e.g., oppositional defiant disorder, conduct disorder), potentially complicating diagnostic clarity in adults.

The second major model from the 1970s was proposed by Virginia I. Douglas. She shifted the focus of the disorder from hyperactivity to deficits in sustained attention and impulse control. While hyperactivity remained clinically visible, Douglas argued that attentional dysregulation and impaired self-regulation constituted the core and more persistent dimensions of the disorder, particularly into adolescence and adulthood. Her model strongly influenced the later DSM transition toward Attention Deficit Disorder ⁸.

Douglas also underscored a critical distinction: not all individuals diagnosed with the disorder required pharmacological treatment. This stance reflected the concerns of the time surrounding stimulant use in children, including uncertainty about their mechanisms of action, potential side effects, and risk of addiction.

Ultimately, her model placed lasting emphasis on the persistence of attentional and impulse control problems into adolescence and beyond. These deficits were not merely academic but also impacted frustration tolerance and social functioning. Douglas concluded that hyperactivity was not the central issue in the long-term course of the disorder; instead, inattention and impulsivity were the symptoms that continued to impair functioning over time ⁸.

d. THE DSM ERA

DSM-III marked a major conceptual shift by replacing the earlier “Hyperkinetic Reaction of Childhood” with Attention Deficit Disorder (ADD), strongly influenced by Douglas’s emphasis on attentional dysfunction and impulse control. Unlike previous frameworks, DSM-III explicitly recognized the possibility that symptoms could persist into adolescence and adulthood, thereby challenging the view of ADHD as an exclusively childhood condition ⁹.

Subsequent DSM revisions progressively refined and operationalized diagnostic criteria, ¹⁰ while also broadening the conceptual scope of the disorder. DSM-IV improved developmental descriptions and symptom consistency requirements. DSM-IV-TR retained these criteria with only minor updates ¹¹. DSM-5 ultimately classified ADHD as a neurodevelopmental disorder, consolidating the idea that attentional and self-regulatory difficulties may persist across the lifespan. This transition significantly expanded the interpretive and diagnostic reach of ADHD into adult populations ¹². Here, ADHD was grouped with Autism Spectrum Disorder (ASD) and Intellectual Disability, under the umbrella of neurodevelopmental disorders-conditions characterized by deviations in normative developmental milestones, either by excess or by deficit. The DSM-5 provided more precise criteria for subtype differentiation and raised the age-of-onset threshold, allowing for greater diagnostic flexibility. ¹³

e. THE CHILDREN GROW UP: THE NEURODIVERSITY MOVEMENT AND ADULT ATTENTION-DEFICIT/HYPERACTIVITY DISORDER

The neurodiversity movement emerged primarily from autistic self-advocacy groups in the mid-1990s^{14,15}. The historical context of this movement is rooted in the broader disability rights movement that emerged in the 1960s^{15,16}. This seminal speech challenged dominant narratives about autism and established the philosophical foundations for autistic self-advocacy¹⁴.

The concept of "neurodiversity" emerged collectively within early online communities of autistic self-advocates, which allowed autistic individuals to connect through the developing Internet^{17,18}. These virtual spaces facilitated the rise of an autistic culture and a self-advocacy movement, affirming autism as a valid way of being^{14,17}.

The neurodiversity movement gradually expanded to encompass a broader array of neurodevelopmental and neurocognitive conditions, most notably ADHD. The increasing inclusion of these diverse neurological differences within the neurodiversity paradigm reflected a broader societal shift toward acknowledging multiple forms of neurological variation as natural and deserving of acceptance and respect within human diversity, rather than as deficits or disorders^{19,20,21}.

3. Medicalization and the history of the Attention-Deficit/Hyperactivity Disorder

The historical evolution of ADHD progressively expanded the range of attentional and behavioral experiences considered susceptible to psychiatric classification. While the recognition of adult ADHD contributed to identifying clinically significant forms of impairment previously overlooked, it also widened the medical interpretation of difficulties related to concentration, productivity, emotional regulation, and self-management in adult life. The DSM-5 classification of ADHD as a neurodevelopmental disorder, together with the growing cultural influence of neurodiversity discourse, reinforced the legitimacy of these diagnostic frameworks. At the same time, such developments intensified broader processes of medicalization and pharmaceuticalization, through which increasingly ordinary dimensions of human behavior became open to clinical and pharmacological intervention.

This trajectory also invites a broader interpretation through the lens of medicalization. The transformation of ADHD from a childhood behavioral syndrome into a lifespan condition affecting adults may be understood not only as a process of diagnostic refinement, but also as part of a broader expansion of psychiatric interpretation into domains of attention, productivity, emotional regulation, and self-management previously understood in educational, moral, occupational, or social terms. In this sense, the historical evolution of ADHD contributed to widening the boundaries of what may be classified as pathological and increasingly subject to clinical and pharmacological intervention. Examining adult ADHD within this framework allows for a more critical assessment of both the genuine benefits of recognition and treatment and the ethical tensions raised by

diagnostic expansion, pharmaceuticalization, and the growing reach of psychiatric categories into ordinary life.

In what follows, we argue that adult ADHD is best understood as a hybrid phenomenon: simultaneously involving legitimate clinical recognition of persistent impairments and broader sociocultural processes of medicalization and diagnostic expansion that reshape how contemporary societies interpret attentional and behavioral differences.

a. ADULT ATTENTION-DEFICIT/HYPERACTIVITY DISORDER AND DIAGNOSTIC EXPANSION

Earlier models largely framed ADHD as a childhood condition associated with hyperactivity, school disruption, and developmental immaturity, as reflected in the DSM-II. By contrast, contemporary diagnostic systems increasingly describe ADHD as a lifespan neurodevelopmental disorder whose symptoms may persist, transform, or become newly visible in adulthood, culminating in its DSM-5 classification within neurodevelopmental disorders^{7,13,22}. This shift expanded not only the age range of diagnosis, but also the kinds of experiences that could be interpreted through the ADHD framework. In childhood, ADHD symptoms are often identified in structured environments where expectations regarding attention, behavioral inhibition, and classroom conduct are externally imposed. In adulthood, however, diagnostic presentations frequently involve difficulties such as procrastination, chronic disorganization, unstable routines, poor time management, emotional impulsivity, occupational underperformance, and relational strain. Many of these experiences exist on a continuum with common human struggles, making the threshold between pathology and normal variation less self-evident than in more overt childhood presentations. As a result, the extension of ADHD into adulthood has widened the interpretive scope of diagnosis^{6,22,23}.

This does not mean that adult ADHD lacks clinical reality or that impairment is merely socially constructed. Many adults experience persistent and substantial difficulties that respond meaningfully to treatment and were historically under-recognized^{7,22}. Nevertheless, diagnostic expansion raises legitimate questions about where boundaries should be drawn, how impairment ought to be defined, and to what extent contemporary institutional demands shape perceptions of disorder. In highly performance-oriented societies, attentional inefficiency may become more visible, costly, and therefore more likely to be medicalized^{24,25}. The rise of adult ADHD may then be understood as involving both recognition and expansion: recognition of genuine longstanding difficulties, and expansion of psychiatric categories into broader areas of adult life previously interpreted in moral, temperamental, educational, or occupational terms. Rising rates of ADHD diagnosis are not reducible to improved recognition alone but are also shaped by institutional and performance pressures. The stigma associated with mental health problems may affect not only those who experience these conditions but also influences the education and attitudes of future healthcare professionals^{26–28}. Moreover, marked regional variation suggests that ADHD diagnosis is partly

mediated by local norms, institutions, and policy environments ²⁹. This dual character helps explain why adult ADHD remains both clinically useful and conceptually contested ³⁰.

b. PHARMACEUTICALIZATION AND THE ATTENTION-DEFICIT/HYPERACTIVITY DISORDER CATEGORY

The growth of ADHD diagnosis and stimulant treatment has been historically reciprocal rather than independent. Stimulants may be sought not only for symptom relief but for adaptation to performance-intensive environments ²⁹. According to the British National Formulary (BNF), psychiatric medications can be broadly classified into several groups, including hypnotics and anxiolytics, drugs used for psychosis and related disorders, antidepressants, and medications prescribed for ADHD and other stimulant-based treatments ^{31,32}. Some of these agents act primarily on the brain and central nervous system, although their effects are often experienced throughout the body.

Psychiatric medications encompass a wide range of substances whose uses have progressively expanded beyond their original indications. Benzodiazepines, for example, initially employed for anxiety and insomnia, are effective sedatives but carry well-known risks of dependence and other adverse effects. Antidepressants act on serotonin and noradrenaline systems, yet their growing use across increasingly diverse conditions has blurred the boundaries between depression, anxiety, grief, and more general psychological distress. These examples illustrate how psychopharmacology has broadened psychiatric intervention while simultaneously raising questions about diagnostic expansion, overprescription, and the medicalization of human suffering ³².

In this latter case, it is important to emphasize that medicalization and the consequent pathologization are not neutral processes, but are shaped by values and cultural standpoints ³³. The growth of Attention Deficit Hyperactivity Disorder diagnoses has gone hand in hand with the expansion of diagnostic categories, generating what has been described as a process of “hyper-medicalization” ²⁴. This process of diagnostic expansion has significant implications for the pathologization of persons, shaping people’s behavior and identity ³³. It also gives rise to a situation in which the limits of pathologization become unclear, as does the point at which a normal range of human performance is redefined as requiring pharmaceutical intervention ³⁴.

c. MEDICALIZATION AND COUNTER-MOVEMENTS

The foregoing has led to the multiplication of mental conditions and their subtypes in ways driven by the commercial objectives of pharmaceutical companies rather than by scientific necessity. This has had unexpected effects on patients, particularly through the labels that these new conditions impose on individuals, in addition to iatrogenic harm—that is, harm produced by a medical treatment as a foreseen but unintended side effect ²⁴. In this regard, patients have used social media both to resist the labeling of persons according to their psychiatric condition and to challenge the psychiatric

establishment ²⁴. The emergence of a specific disorder in a given time and place becomes possible through a historical and cultural context that makes it intelligible. Once it appears, it “takes on a life of its own,” affecting people’s lives insofar as a particular diagnosis becomes part of a person’s identity ²⁴.

A significant consequence of this perspective can be seen in the decision of some scientists to move away from standard classifications of mental disorders such as the Diagnostic and Statistical Manual of Mental Disorders. Thus, neuroscientists associated with the National Institute of Mental Health have indicated that they would no longer use the DSM as their primary theoretical framework ^{35,36}. These researchers argue that it is more productive to focus on understanding the biological substrates of experimentally accessible psychological phenomena than to continue searching for neurobiological, neurochemical, and pharmacological explanations tied to conventional mental health diagnoses and their drug treatments.

Instead, the National Institute of Mental Health proposed a new framework known as Research Domain Criteria. This system focuses on fundamental psychological and neurobiological processes—such as positive and negative valence systems, perception, working memory, affiliation, and attachment—rather than on the DSM’s clinical categories ³⁷. This shift represents an epistemological rupture: neuroscientific research moved from seeking chemical or pharmacological correlates for diagnoses such as Schizophrenia, Depression, or Bipolar Disorder toward investigating the biological substrates of experimentally tractable psychological phenomena such as motivation or emotional regulation. Middleton observes that this transition weakens the assumption that psychopharmacology can provide precise treatments for clinically defined diseases, since it undermines the very idea that such categories correspond to stable biological entities ^{32(pp78-79)}.

d. ADULT ATTENTION-DEFICIT/HYPERACTIVITY DISORDER, MEDICALIZATION, AND PHARMACEUTICALIZATION

The historical evolution of ADHD contributed to progressively expanding the range of attentional and behavioral experiences interpreted through psychiatric categories. While many adults experience persistent and clinically significant difficulties that were historically under-recognized and may respond meaningfully to treatment, the extension of ADHD into adulthood also raises important questions regarding the limits of diagnosis and the definition of impairment. In highly performance-oriented societies, difficulties related to concentration, productivity, emotional regulation, and self-management become increasingly visible and socially costly, making them more susceptible to medical interpretation. The emergence of adult ADHD may therefore be understood as involving both recognition and expansion: recognition of genuine longstanding forms of suffering, and expansion of psychiatric frameworks into domains of adult life previously interpreted in moral, temperamental, educational, or occupational terms.

This process has been closely tied to pharmaceuticalization. The growth of ADHD diagnoses and stimulant treatment has historically been reciprocal rather than independent, as psychopharmacological interventions increasingly shape how attentional and behavioral difficulties are conceptualized and managed. In this context, stimulant medication may be sought not only for symptom relief, but also for adaptation to performance-intensive environments. Such developments raise broader ethical concerns regarding diagnostic expansion, overprescription, and the redefinition of ordinary variations in human performance as conditions requiring pharmaceutical intervention. Moreover, these processes are not ethically or culturally neutral: the expansion of psychiatric categories is shaped by institutional pressures, social values, and commercial interests that influence how suffering, productivity, and normality are understood. Contemporary criticisms of DSM-based psychiatry and the growing skepticism toward stable diagnostic entities further illustrate the conceptual and ethical tensions surrounding the medicalization of adult attention and self-regulation.

Conclusions

The historical evolution of ADHD reflects more than the progressive refinement of a psychiatric diagnosis. It also illustrates how contemporary societies increasingly interpret attentional, behavioral, and self-regulatory difficulties through medical and pharmacological frameworks. The transformation of ADHD from a childhood behavioral syndrome into a lifespan neurodevelopmental condition expanded not only the

diagnostic scope of psychiatry, but also the range of ordinary human experiences susceptible to clinical interpretation and intervention.

Understanding adult ADHD as a hybrid phenomenon allows for a more balanced ethical assessment of its contemporary status. On the one hand, the recognition of adult ADHD has contributed to identifying clinically significant forms of suffering that were historically neglected and may respond meaningfully to treatment. On the other hand, the expansion of diagnostic categories into broader domains of adult life raises important conceptual and ethical questions regarding the limits of pathology, the influence of performance-oriented social environments, and the growing role of pharmaceutical intervention in shaping contemporary forms of subjectivity and self-regulation.

From this perspective, medicalization and pharmaceuticalization should not be understood as ethically neutral processes, but as historically and culturally situated frameworks that shape how societies define normality, productivity, and impairment. The case of adult ADHD therefore illustrates a broader tension within contemporary psychiatry: how to recognize genuine forms of suffering without collapsing the distinction between pathology and ordinary human variation. A critical ethical approach to adult ADHD requires maintaining both dimensions simultaneously—acknowledging the legitimacy of clinical impairment while critically examining the conceptual expansion of psychiatric categories and their increasing influence over everyday life.

References

1. Laufer MW, Denhoff E, Solomons G. Hyperkinetic impulse disorder in children's behavior problems. *Journal of Attention Disorders*. 2011;15(8):620-625. doi:10.1177/1087054711413043
2. Laufer MW, Denhoff E. Hyperkinetic behavior syndrome in children. *The Journal of Pediatrics*. 1957;50(4):463-474. doi:10.1016/S0022-3476(57) 80257-1
3. Conners CK. History of attention-deficit hyperactivity disorder (ADHD). In: *Attention-Deficit Hyperactivity Disorder in Adults and Children*. Cambridge University Press; 2015:1-15. doi:10.1017/CBO9781139035491.002
4. American Psychiatric Association. *Diagnostic and Statistical Manual of Mental Disorders*. 2nd ed. American Psychiatric Association; 1968.
5. Wender PH. The minimal brain dysfunction syndrome in children. I. The syndrome and its relevance for psychiatry. II. A psychological and biochemical model for the syndrome. *The Journal of Nervous and Mental Disease*. 1972;155(1):55-71. doi:10.1097/00005053-197207000-00007
6. Wood DR, Reimherr FW, Wender PH, Johnson GE. Diagnosis and treatment of minimal brain dysfunction in adults: a preliminary report. *Archives of General Psychiatry*. 1976;33(12):1453-1460. doi:10.1001/ARCHPSYC.1976.01770120057005
7. Faraone SV, Bellgrove MA, Brikell I, et al. Attention-deficit/hyperactivity disorder. *Nature Reviews Disease Primers*. 2024;10(1):1-21. doi:10.1038/s41572-024-00495-0
8. Douglas VI. Stop, look and listen: The problem of sustained attention and impulse control in hyperactive and normal children. *Canadian Journal of Behavioural Science / Revue Canadienne Des Sciences Du Comportement*. 1972;4(4):259-282. doi:10.1037/H0082313
9. American Psychiatric Association. *Diagnostic and Statistical Manual of Mental Disorders (DSM III)*. 3rd ed. American Psychiatric Publishing, Inc.; 1980.
10. American Psychiatric Association. *Diagnostic and Statistical Manual of Mental Disorders*. 3rd, revised eds. American Psychiatric Association; 1987.
11. American Psychiatric Association. *Diagnostic and Statistical Manual of Mental Disorders: DSM IV-TR*. American Psychiatric Publishing; 2000.
12. American Psychiatric Association. *Diagnostic and Statistical Manual of Mental Disorders (DSM-IV)*. American Psychiatric Publishing, Inc.; 1994.
13. American Psychiatric Association. *Diagnostic and Statistical Manual of Mental Disorders (DSM-5)*. American Psychiatric Publishing, Inc.; 2013.
14. Leadbitter K, Buckle KL, Ellis C, Dekker M. Autistic Self-Advocacy and the Neurodiversity Movement: Implications for Autism Early Intervention Research and Practice. *Front Psychol*. 2021;12. doi:10.3389/fpsyg.2021.635690
15. Stenning A, Bertilsdotter-Rosqvist H. Neurodiversity studies: mapping out possibilities of a new critical paradigm. *Disabil Soc*. 2021;36(9):1532-1537. doi:10.1080/09687599.2021.1919503
16. Shaw S. When I say neurodiversity paradigm. *Medical Education*. Published online 2025.
17. Botha M, Chapman R, Giwa Onaiwu M, Kapp SK, Stannard Ashley A, Walker N. The neurodiversity concept was developed collectively: An overdue correction on the origins of neurodiversity theory. *Autism*. 2024;28(6):1591-1594. doi:10.1177/13623613241237871
18. Stenner P, Pearson A, Kapp S, et al. Using Q Methodology, a Group of Neurodivergent Neurodiversity Researchers Ask: What is the Neurodiversity Movement and what should it do? *Neurodiversity*. 2025;3:27546330251329625. doi:10.1177/27546330251329625
19. Fletcher-Watson S. Transdiagnostic research and the neurodiversity paradigm: Commentary on the transdiagnostic revolution in neurodevelopmental disorders by Astle et al. *Journal of Child Psychology and Psychiatry*. 2022;63(4):418-420. doi:10.1111/jcpp.13589
20. Smith M. Hyperactive Around the World? *Social History of Medicine*. Published online 2017.
21. Rosqvist HB, Hultman L, Wiklund SÖ, Nygren A, Storm P, Sandberg G. Naming ourselves, becoming neurodivergent scholars. *Disability & Society*. 2025;40(1):128-147. doi:10.1080/09687599.2023.2271155
22. Faraone SV, Banaschewski T, Coghill D, et al. The World Federation of ADHD International Consensus Statement: 208 Evidence-based conclusions about the disorder. *Neuroscience & Biobehavioral Reviews*. 2021;128:789-818. doi:10.1016/j.neubiorev.2021.01.022
23. Barkley RA. *Attention-Deficit Hyperactivity Disorder: A Handbook for Diagnosis and Treatment, 4th Ed.* (Barkley RA, ed.). The Guilford Press; 2015.
24. Charland LC. Why Psychiatry Should Fear Medicalization. In: Fulford KWM, Davies M, Gipps R, et al., eds. *The Oxford Handbook of Philosophy and Psychiatry*. Oxford University Press; 2013:159-165. doi:10.1093/oxfordhb/9780199579563.013.0013
25. Conrad P, Bergey MR. The impending globalization of ADHD: Notes on the expansion and growth of a medicalized disorder. *Social Science & Medicine*. 2014;122:31-43. doi:10.1016/j.socscimed.2014.10.019
26. Corrigan P, Watson A. Understanding the impact of stigma on people with mental illness. *World Psychiatry*. 2002;1(1):16-20.
27. Kvaale EP, Haslam N, Gottdiener WH. The 'side effects' of medicalization: A meta-analytic review of how biogenetic explanations affect stigma. *Clinical Psychology Review*. 2013;33(6):782-794. doi:10.1016/j.cpr.2013.06.002
28. Masedo A, Grandón P, Saldivia S, et al. A multicentric study on stigma towards people with mental illness in health sciences students. *BMC Medical Education*. 2021;21(1):324. doi:10.1186/s12909-021-02695-8
29. Hinshaw SP, Scheffler RM. *The ADHD Explosion: Myths, Medication, Money, and Today's Push for Performance*. 2014.
30. Epstein JN, Loren REA. Changes in the Definition of ADHD in DSM-5: Subtle but Important.

- Neuropsychiatry* (London). 2013;3(5):455-458. doi:10.2217/npv.13.59
31. Joint Formulary Committee, ed. *British National Formulary*. 89th ed. BMJ Publishing Group Ltd and Pharmaceutical Press; 2025.
32. Middleton H. *Psychiatry Reconsidered: From Medical Treatment to Supportive Understanding*. Palgrave Macmillan; 2015. doi:10.1057/9781137384904
33. Sadler JZ. Values in Diagnosing and Classifying Psychopathology. In: Sadler JZ, Fulford KWM, van Staden W (C. W), eds. *The Oxford Handbook of Psychiatric Ethics*, Vol. 2. Oxford University Press; 2015:753-778. doi:10.1093/oxfordhb/9780198732372.013.20
34. Arancibia-Collao F, Martínez-Villavicencio C. Conceptualización del Trastorno de Déficit Atencional e Hiperactividad del DSM-5 y la prescripción de psicoestimulantes de uso no médico: una exploración a un dilema bioético. *Medicina y Ética*. 2026;37(2):614-674. doi:10.36105/mye.2026v37n2.07
35. American Psychological Association. NIMH funding to shift away from DSM categories. <https://www.apa.org>. 2013. Accessed October 27, 2025. <https://www.apa.org/monitor/2013/07-08/nimh>
36. National Institute of Mental Health. Revolutionizing the Study of Mental Disorders - National Institute of Mental Health. March 27, 2024. Accessed October 27, 2025. <https://www.nimh.nih.gov/news/science-updates/2024/revolutionizing-the-study-of-mental-disorders>
37. National Institute of Mental Health. About RDoC - National Institute of Mental Health (NIMH). Accessed October 27, 2025. <https://www.nimh.nih.gov/research/research-funded-by-nimh/rdoc/about-rdoc>